



Wearable Cardioverter Defibrillator Vest

History

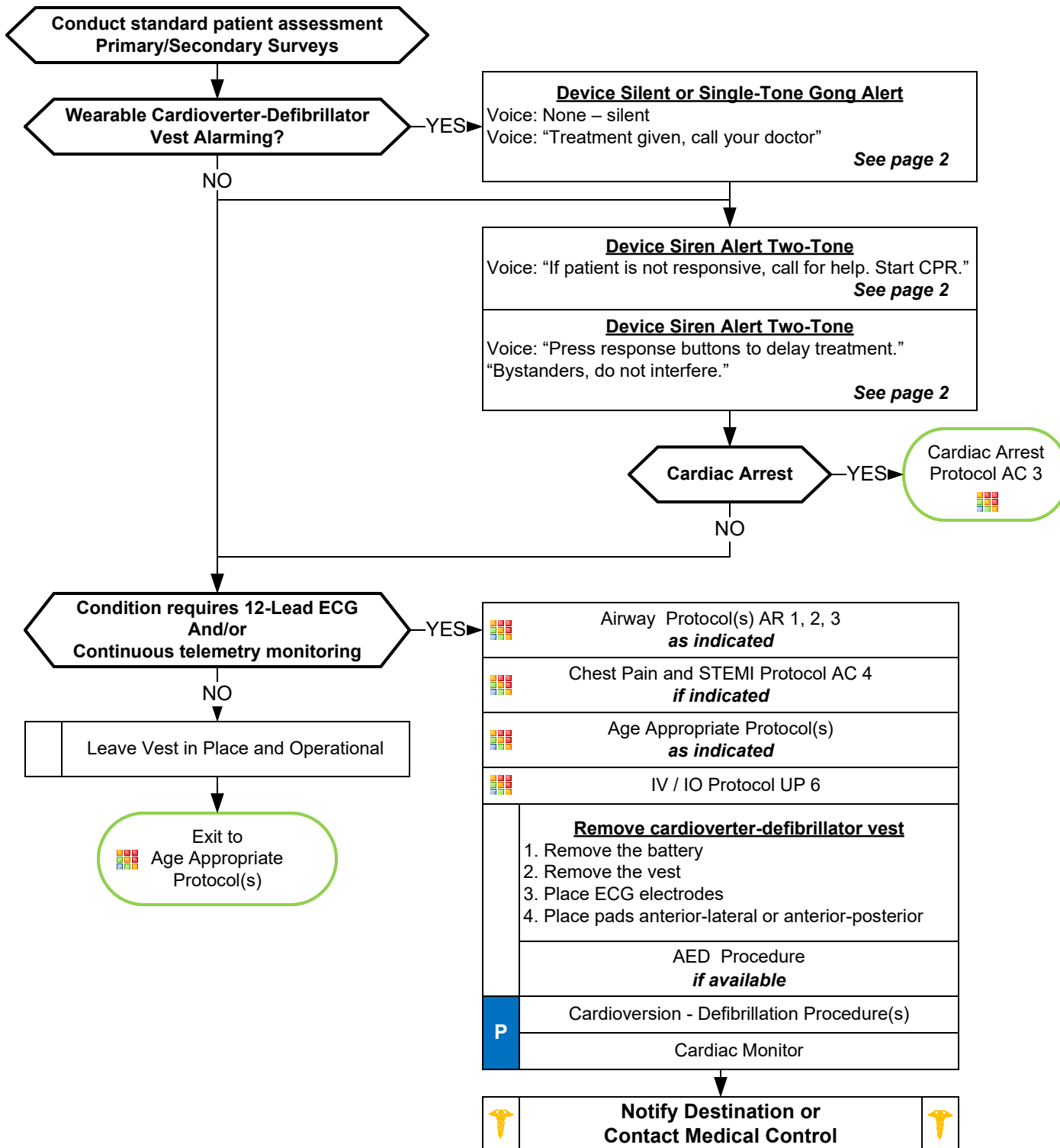
- SAMPLE
- Known risk for Sudden Cardiac Death
- Risk for life-threatening arrhythmia
- No implanted defibrillator
- Heart failure – cardiomyopathy
- Decreased ejection fraction

Signs and Symptoms

- Chest pain, dyspnea
- Palpitations
- Received shock from vest
- Poor capillary refill / skin color
- AMS or decreased mental status

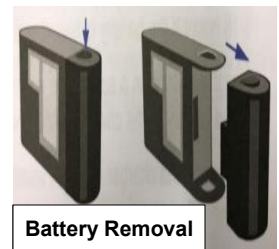
Differential

- See Reversible Causes below
- Arrhythmia
- Infection/Sepsis
- Hypovolemia
- Cardiac arrest
- Hemorrhage





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Battery Removal

Response button

Pearls

- **Recommended exam:** Mental status, skin color, capillary refill, peripheral pulses, blood pressure.
- **Wearable Cardioverter-Defibrillator Vest:**
 - Device is preparing to delivery a shock to the patient:**
Before device delivers a shock, it tests to see if patient is conscious – voice prompt instructs patient to press the “response” button (see diagram above).
Only the patient should press the “response” button.
Once a treatable arrhythmia is detected it takes between 25 and 60 seconds to deliver the shock.
- **Audible and tactile warning system:**
The device will provide a vibration, a siren tone, and voice prompts to check if the patient is conscious and give them an opportunity to press the “response” button to abort a shock.
See audible warning system above.
- **Reasons for use:**
Currently only device on the market is the Zoll LifeVest.
Worn by patients at risk of sudden cardiac arrest or risk of abnormal and/or lethal arrhythmia.
- **Blue gel on the patient’s skin from the device:**
Electrode pads release a blue gel prior to treatment to improve shock conduction and reduce burning.
Do not remove the gel if the vest is left in place during treatment.
Remove gel if vest is removed for prehospital care.
- **Shock to providers:**
Do not touch the patient when the device is instructing you that a shock will be delivered.
Providers can be shocked by the device during energy delivery if provider is touching the patient.
- **Removing the device for prehospital care:**
The device should only be removed when ECG monitor and defibrillator is available.
Continuous ECG monitoring and electrode pads should be in place when vest is removed.
- **Defibrillation/cardioversion with vest in place:**
Disconnect the device from the vest before you deliver a cardioversion or defibrillation
- **Transcutaneous Pacing:**
May be utilized with vest in place – disconnect the device from the vest before you perform transcutaneous pacing.